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Owner Courtney
 Doellner: Clinical Pharmacist
 Specialist– Pediatric Medication
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 Applicability UMHS Clinical
 References Guideline

Pharmacy Services IV Guidelines for Pediatric Intensive Care Unit (PICU) / CES Guideline

DRUG – GENERIC NAME [TRADE NAME®]	LOAD/BOLUS DOSING	USUAL DOSING RANGE	NURSING TITRATION INSTRUCTIONS	COMMENTS	ADMINISTRATION CONCERNS
ALPROSTADIL (PGE, PROSTIN, PROSTAGLANDIN) <i>*Used for PDA patency or graft patency (liver tx)</i>	NONE	0.03-0.2 mcg/kg/ min Hard max: 0.4 mcg/kg/min Transplant: 10-40 mcg/hr based on patient weight	DO NOT TITRATE	Transplant dosing is different; refer to liver transplant protocol in OTIS	Central line administration - If <1 mL/hr, then D10W (or D5W) WITHOUT heparin should be co-infused - If >1 mL/hr, no fluids needed for co-infusion
AMINOPHYLLINE <i>*IV form of theophylline</i>	Loading: 5-8 mg/kg IV given over 30 minutes	0.5-1.5 mg/kg/hr Hard max: 1.5 mg/kg/hr	DO NOT TITRATE	- Follow theophylline levels 1 hour post-load, then Q12-24hr - Therapeutic: 10-15 mcg/mL - Toxicity > 20 mcg/mL	
AMIODARONE	Loading: <50kg: 5mg/ kg IV (max 150mg/dose) over 60 minutes May repeat loading dose (max 15 mg/	<50kg or <18 yoa: 5-15 mcg/kg/min Hard max: 20 mcg/kg/min ≥50kg or ≥18 yoa: 1 mg/min x 6hrs,	DO NOT TITRATE		MUST BE FILTERED with 0.22micron filter - Tubing good for 96 hours - Concentrations

	kg/day or 450 mg/day) ≥50kg: 150mg/dose IV over 60 minutes ; may repeat (Max 450 mg/day)	then 0.5 mg/min Hard max: 2 mg/min			greater than 2 mg/mL must go via central line • Risk of phlebitis and extravasation with peripheral use – call Pharmacy (48208) if extravasation occurs • Loading dose MUST be given over 60 minutes to avoid hypotension • Diluents: D5W (preferred)
ARGATROBAN	Loading: ONLY used in cath lab or to prime ECMO circuit	0.1-3 mcg/kg/min ECMO: 0.5-5 mcg/kg/min Hard max: 10 mcg/kg/min	DO NOT TITRATE	- Follow aPTTs or anti- IIa levels - ECMO follows ACTs - Requires hematology approval	
AMINOCAPROIC ACID [AMICAR®]	Loading: 50-100 mg/kg IV (max 5000 mg/dose) given over 15-60 minutes	<50kg: 25-30 mg/kg/hr Hard max: 100 mg/kg/hr Adult order: 1 g/hr Soft max: 1.5 g/hr	DO NOT TITRATE	- Pediatric dosing in mg/kg/hr - Adult dosing in g/hr - Usually reserved for patients on ECMO with bleeding concerns	
BUMETANIDE [BUMEX®]	NONE	0.01-0.05 mg/kg/hr Hard max: 0.08 mg/kg/hr (or 5 mg/hr)	DO NOT TITRATE		No diluent – this is straight drug
CALCIUM CHLORIDE	NONE	2.5-20 mg/kg/hr Hard max: 30 mg/kg/hr CRRT Calcium Infusion: per CRRT Citrate protocol	DO NOT TITRATE Exception: For CRRT calcium infusion, titrate per CRRT Citrate protocol		CENTRAL LINE administration ONLY - Vesicant – if extravasation occurs, call Pharmacy (48208)
CISATRICURIUM [NIMBEX®]	Loading: may give 0.1 mg/kg and then	1-10 mcg/kg/min Hard max: 13 mcg/kg/min	Titrate by 0.5 mcg/kg/min every 30-60	Neuromuscular blockade Monitor with	The 2 mg/mL has no diluent

	initiate continuous infusion Bolus: may give 0.1 mg/kg with each infusion increase		minutes until ordered Train-of-Four or max ordered dose achieved	peripheral nerve stimulation (Train-of-Four) • Obtain baseline TOF	– this is straight drug
DEXMEDETOMIDINE [PRECEDEX®]	Due to potential side effects, loading doses are not currently recommended	0.1 – 2 mcg/kg/hr Hard max: 3 mcg/kg/hr	Titrate by 0.1-0.2 mcg/kg/hr every 30 to 60 minutes until ordered sedation or max ordered dose achieved	• Common side effects: bradycardia and hypotension	
DILTIAZEM [CARDIZEM®]	Loading: 0.25 mg/kg IV given over 2 minutes (max 25 mg); after 15 minutes if cardiac rhythm response is not achieved, prescriber may order 2 nd dose of 0.35 mg/kg (max 25 mg)	<50kg: 0.05-0.2 mg/kg/hr Hard max: 0.3 mg/kg/hr ≥50kg: 5-15 mg/hr Hard max: 20 mg/hr	DO NOT TITRATE	• Pediatric dosing in mg/kg/hr • Adult dosing in mg/hr	• Administer as a continuous IV infusion with the use of an infusion pump with continuous ECG monitoring; response may persist for several hours after infusion is discontinued
DOBUTAMINE	NONE	2.5-20 mcg/kg/min Hard max: 25 mcg/kg/min	Titrate by 2.5-5 mcg/kg/min every 5 minutes until ordered parameter or max ordered dose achieved		• Vesicant – if extravasation occurs, call Pharmacy (48208)
DOPAMINE	NONE	2.5-20 mcg/kg/min Hard max: 25 mcg/kg/min	Titrate by 2.5 mcg/kg/min every 5 minutes until ordered parameter or max ordered dose achieved	• Low dose: 1-5 mcg/kg/min; increase renal blood flow and urine output • Intermediate dose: 5-15 mcg/kg/min; increase heart rate, CO and BP • High dose: >15 mcg/kg/min; increase BP, vasoconstriction	• Vesicant – if extravasation occurs, call Pharmacy (48208)
EPINEPHRINE HYDROCHLORIDE	NONE	0.01-0.1 mcg/kg/min Hard max: 4	Titrate by 0.01-0.02 mcg/kg/min every 5		• Vesicant – if extravasation

		mcg/kg/min	minutes until ordered parameter or max ordered dose achieved		occurs, call Pharmacy (48208)
EPOPROSTANOL [FLOLAN®]	NONE	2-20 ng/kg/min Hard max: 125 ng/kg/min Starting dose: 2-3 ng/kg/min; increased based on patient response Typical dose increase by 1-2 ng/kg/min 1 ng (nanogram) = 0.001 mcg	DO NOT TITRATE		• Flolan syringes must be changed at least every 72 hours, when the drug syringe expires or when the tubing needs changing per unit policy, which ever comes sooner; protect from light • MUST BE FILTERED with 0.22 micron filter • Flolan is NOT COMPATIBLE with any other meds or IV fluids; run in dedicated line • DO NOT stop, pause, or interrupt infusion • Diluent: special Flolan diluent
ESMOLOL [BREVIBLOC®]	Loading: 50-500mcg/kg given over 1 minute	50-250 mcg/kg/min Hard max: 800 mcg/kg/min	DO NOT TITRATE		• No Diluent - straight drug
FENOLDOPAM	NONE	0.1-0.8 mcg/kg/min Hard max: 0.8 mcg/kg/min	DO NOT TITRATE		
FENTANYL [SUBLIMAZE®]	Loading: 0.5-5 mcg/kg IV given slow IV push Bolus: usually equal to infusion rate given q1-4h prn; may give 1-2 mcg/kg	<50kg: 0.5-3 mcg/kg/hr Hard Max: 10 mcg/kg/hr ≥50kg: 25-200 mcg/hr Hard Max: 400 mcg/hr	DO NOT TITRATE	• Pediatric dosing in mcg/kg/hr • Adult dosing in mcg/hr	• Pushing IV fentanyl too quickly may result in chest wall rigidity • The 50 mg/mL concentration has no diluent – straight drug

	slow IV push with each infusion increase Administration rates loading/ bolus doses: <5mcg/kg = 3-5 minutes ≥5mcg/kg = 5-10 minutes				
FUROSEMIDE [LASIX®]	NONE	<50kg: 0.05-1 mg/kg/hr Hard max: 1.5 mg/kg/hr ≥50kg: 5-40 mg/hr Hard max: 160 mg/hr	DO NOT TITRATE	- Pediatric dosing in mg/kg/hr - Adult dosing in mg/hr	The 10 mg/mL concentration has no diluent – this is straight drug
HEPARIN	Loading: 50-100 units/kg IV given over 15 minutes; per MD request ECMO Loading: 100 units/kg IV x 1	Initial: 10-20 units/kg/hr Hard max: 75 units/kg/hr ECMO: 10-40 units/kg/hr Hard max: 100 units/kg/hr	DO NOT TITRATE	- Monitor aPTT or anti-Xa - Doses will be adjusted per levels - ECMO follows ACTs	
HYDROMORPHONE	Bolus: usually equal to infusion rate given q1-4h prn	<50kg: 5-20 mcg/kg/hr Hard max: 45 mcg/kg/hr ≥50kg: 0.25-3 mg/hr Hard max: 5 mg/hr	<50kg: titrate by 1 mcg/kg/hr every 4 hours until ordered parameter or max ordered dose achieved >50kg: titrate by 0.1 mg/hr every 4 hours ordered parameter or max ordered dose achieved	- Pediatric dosing in mcg/kg/hr - Adult dosing in mg/hr - Note: hydromorphone is ~7-10x as potent as morphine (i.e. morphine 200 mcg/kg/hr ≈ hydromorphone 20-28 mcg/kg/hr)	Use is restricted for sedation in mechanically ventilated opioid tolerant patients
INSULIN REGULAR	NONE	DKA: 0.05-0.1 units/kg/hr HYPERGLYCEMIA of critical illness: <50kg: Initiate at 0.01-0.1 units/kg/hr Hard max: 0.4 units/kg/hr ≥50kg: Initiate at	DO NOT TITRATE	- Pediatric dosing in units/kg/hr - Adult dosing in units/hr - For insulin infusion guidelines on pediatric diabetic patients see Pediatric Endocrinology	MONITOR GLUCOSE LEVELS

		1-5 units/hr [1 unit/mL concentration ONLY] Hard max: 30 units/hr		Guidelines for DKA	
ISOPROTERENOL [ISUPREL®]	NONE	<50kg: 0.01-2 mcg/kg/min Hard max: 2 mcg/kg/min ≥50kg: 2-10 mcg/min Hard max: 11 mcg/min	DO NOT TITRATE	- Pediatric dosing in mcg/kg/min - Adult dosing in mcg/min	- Concentrations greater than 0.01 mg/mL must go via central line - Vesicant – if extravasation occurs, call Pharmacy (48208)
KETAMINE [KETALAR®]	Bolus: may give 0.5-2 mg/kg IV over 1-3 minutes	5-20 mcg/kg/min Hard max: 80 mcg/kg/min	titrate by 5 mcg/kg/min every 2 hours until ordered sedation or max ordered dose achieved	Must use PCTU/ PICU profile in Alaris device	
LABETOLOL	Bolus: 0.2-1 mg/kg (max 40 mg/dose); may administer over 2 minutes as long as under max of 10 mg/min	<50kg: 0.25-3 mg/kg/hr Hard max: 3 mg/kg/hr ≥50kg: 0.5-2 mg/min Hard max: 6 mg/min	DO NOT TITRATE	- Pediatric dosing in mg/kg/hr - Adult dosing in mg/min	Concentrations greater than 2 mg/mL must go via central line
LEVOTHYROXINE (T4 – GIFT OF LIFE) [SYNTHROID®]	Loading: 0.8-5 mcg/kg IV; administered over 30 minutes	<50kg: 0.8-1.4 mcg/kg/hr Hard max: 1.4 mcg/kg/hr ≥50kg: 1-20 mcg/hr Hard max: 20 mcg/hr	TITRATE BASED ON ORDER	- Pediatric dosing in mcg/kg/hr - Adult dosing in mcg/hr	Age based dosing available in Lexicomp
LIDOCAINE	Cardiac Loading: 1 mg/kg IV given over 2 minutes followed by continuous IV infusion; may administer second bolus if delay between initial bolus and start of infusion is >15 minutes	Cardiac: <50 kg: 20-50 mcg/kg/min Hard max: 80 mcg/kg/min ≥50kg: 0.5-4 mg/min Hard max: 5 mg/min Chronic pain: 15-50 mcg/kg/min (max starting rate 1600 mcg/min), infusion typically	DO NOT TITRATE	- Pediatric dosing in mcg/kg/min - Adult dosing in mg/min	- Concentrations greater than 4 mg/mL must go via central line - Check lidocaine levels frequently as requested by MD, space to Q24hr once therapeutic - Therapeutic: 1.5-5 mcg/mL

		maintained for 24-48 h per policy; aim for lowest dose Hard max: 50 mcg/kg/min			
MAGNESIUM SULFATE	Loading: may give 25-75 mg/kg (max 2000 mg/dose) over 30-60 minutes	20 mg/kg/hr Hard max: 50 mg/kg/hr	DO NOT TITRATE	- Check magnesium level after loading dose and every 6 hours while on infusion - Goal 3-4 mg/dL	
MIDAZOLAM [VERSED®]	Loading: 0.05-0.2 mg/kg IV given over 2 minutes Bolus: May give 0.05-0.2 mg/kg IV over 2 minutes with each infusion increase	<50kg: 10-150 mcg/kg/hr Hard max: 300 mcg/kg/hr ≥50kg: 1-10 mg/hr Hard max: 20 mg/hr	<50kg: titrate by 10-20 mcg/kg/hr every 60 minutes until ordered sedation or max ordered dose achieved >50kg: titrate by 0.5-1 mg/hr every 60 minutes until ordered sedation or max ordered dose achieved	- Pediatric dosing in mcg/kg/hr - Adult dosing in mg/hr HIGH DOSE MIDAZOLAM – Seizure EEG Burst Suppression Order Set » Concentration: 1 mg/ml » DO NOT TITRATE Dose: 50-3000 mcg/kg/hr No hard max	
MILRINONE	NONE	0.3-0.75 mcg/kg/min Hard max: 1 mcg/kg/min	DO NOT TITRATE	125mg/250mL (0.5 mg/mL) is restricted to continuation of home therapy ONLY	
MORPHINE SULFATE	Loading: 0.1-0.3 mg/kg IV given over 5 minutes Bolus: usually equal to infusion rate given q1-4h prn	<50kg: 10-150 mcg/kg/hr Hard max: 300 mcg/kg/hr ≥50kg: 1-5 mg/hr Hard max: 20 mg/hr	<50kg: titrate by 10-20 mcg/kg/hr every 60 minutes until ordered parameter or max ordered dose achieved >50kg: titrate by 0.5-1 mg/hr every 60 minutes ordered parameter or max ordered dose achieved	- Pediatric dosing in mcg/kg/hr - Adult dosing in mg/hr	Monitor for histaminergic/allergic response
NALOXONE	NONE	For opioid	DO NOT		

(NARCAN®)		reversal: 20-40 mcg/kg/hr Hard max: 40 mcg/kg/hr Other Indications: 0.1-0.25 mcg/kg/hr Hard max: 0.25 mcg/kg/hr	TITRATE		
NICARDIPINE	NONE	<50 kg: 0.5-5 mcg/kg/min Hard max: 6 mcg/kg/min ≥50kg or ≥18 yoa: 5-15 mg/hr Hard max: 15 mg/hr	<50kg: titrate by 0.2 mcg/kg/min every 5 minutes ordered parameter or max ordered dose achieved ≥50kg: titrate by 2.5 mg/hr every 5 minutes ordered effects or max ordered dose achieved	- Pediatric dosing in mcg/kg/min - Adult dosing in mg/hr	- Concentrations greater than 0.2 mg/mL must go via central line - Change peripheral infusion site every 12 hours to avoid irritation - Risk of phlebitis and extravasation with peripheral use – call Pharmacy (48208) if extravasation occurs
NITROGLYCERIN	NONE	<50 kg: 0.5-5 mcg/kg/min Hard max: 20 mcg/kg/min ≥50kg or ≥18 yoa: 5-200 mcg/min Hard max: 210 mcg/min	DO NOT TITRATE	- Pediatric dosing in mcg/kg/min - Adult dosing in mcg/min	- Concentrations greater than 0.2 mg/mL must go via central line - Use special IV tubing (low sorbing tubing) available in nursing clean room
NITROPRUSSIDE [NIPRIDE®]	NONE	0.3-10 mcg/kg/min Hard max: 11 mcg/kg/min Doses > 400 mcg/min not recommended due to increase risk thiocyanate toxicity, with little added benefit	DO NOT TITRATE	- Check cyanide/thiocyanate level every 24 hours to monitor for cyanide toxicity - Cyanide toxicity: >2 mcg/mL - Thiocyanate toxicity: >35 mcg/mL	- Concentrations greater than 0.5 mg/mL must go via central line Protect syringe from light – do not need to cover IV tubing
NOREPINEPHRINE BITARTRATE	NONE	0.01-0.1 mcg/kg/min	Titrate by 0.01-0.02 mcg/		Concentrations

[LEVOPHED®]		Hard max: 2 mcg/kg/min	kg/min every 5 minutes ordered parameter or max ordered dose achieved		greater than 0.01 mg/mL must go via central line Vesicant – if extravasation occurs, call Pharmacy (48208)
OCTREOTIDE	Load: for GI bleed, may administer 1-2 mcg/kg IV bolus over 15-30 minutes; followed by continuous infusion	1-10 mcg/kg/hr Hard max: 11 mcg/kg/hr ≥50 kg or ≥18 yoa: 25-50 mcg/hr Hard max: 75 mcg/hr	DO NOT TITRATE	- Pediatric dosing in mcg/kg/hr - Adult dosing in mcg/hr	
PANTOPRAZOLE [PROTONIX®]	Loading: 1-2 mg/kg IV (max 80 mg/dose) given over 5 minutes	<50kg: 0.1-0.2mg/kg/hr Hard max: 0.5 mg/kg/hr ≥50 kg: 5-10 mg/hr Hard max: 12 mg/hr	DO NOT TITRATE	- Pediatric dosing in mg/kg/hr - Adult dosing in mg/hr	Should be used for GI bleeding only
PENTOBARBITAL [NEMBUTAL®]	Loading: 1-5 mg/kg IV given over 20 minutes Bolus: may give 1-2 mg/kg IV over 20 minutes with each infusion increase	0.25-7.5 mg/kg/hr Hard max: 15 mg/kg/hr	Titration orders available in MiChart to goal FLACC, Verbal numeric pain rating, RASS score		- Infusion, tubing, and stopcock needs to be changed Q24hr - Does NOT need to be filtered
PHENYLEPHRINE	Bolus: 5-20 mcg/kg/dose every 10 to 15 minutes as needed	0.05-5 mcg/kg/min Hard max: 5 mcg/kg/min ≥50 kg: 50-300 mcg/min Hard max: 300 mcg/min	Titration available according to physician instructions per MiChart	- Pediatric dosing in mcg/kg/min - Adult dosing in mcg/min	- Vesicant – if extravasation occurs, call Pharmacy (48208) - Central line administration preferred
PROCAINAMIDE	Loading: <50kg: 7-15 mg/kg (max 1500 mg/dose) over 30-60 minutes ≥50kg: 10-17 mg/kg at a rate of 20-50	<50kg: 20-80 mcg/kg/min, max of 2000 mg in 24 hr Hard max: 100 mcg/kg/min ≥50kg: 1-4 mg/min Hard max: 6 mg/	DO NOT TITRATE	- Pediatric dosing in mcg/kg/min - Adult dosing in mg/min - Check Procainamide and NAPA levels 6 to 12 hours after start of	

	mg/minute or 100 mg every 5 minutes until arrhythmia is controlled, hypotension occurs or QRS complex widens by 50% of its original width	min		infusion per MD - PROC toxicity: >12 mcg/mL - NAPA toxicity: >40 mcg/mL	
PROPOFOL [DIPRIVAN®]	Loading/Bolus: 0.5-3 mg/kg	10-150 mcg/kg/min Hard max: 150 mcg/kg/min	DO NOT TITRATE	Refer to: Pediatric Propofol Policy (07-01-033)	Discard infusion AND tubing every 12 hours No diluent – straight drug - Vesicant – if extravasation occurs, call Pharmacy (48208)
ROCURONIUM	Loading: may give 1 mg/kg IV over 5-10 seconds prior to initiating continuous infusion Bolus: May give 1 mg/kg IV over 5-10 seconds with each infusion increase	5-12 mcg/kg/min Hard max: 12 mcg/kg/min	Titrate by 1 mcg/kg/min every 30-60 minutes until ordered Train-of-Four or max ordered dose achieved	Neuromuscular blockade - Monitor with peripheral nerve stimulation (Train-of-Four) - Obtain baseline TOF	Vesicant – if extravasation occurs, call Pharmacy (48208)
SILDENAFIL	NONE	Typical starting dose: 0.067 mg/kg/hr (= 1.67 mg/kg/DAY) Dosing range: 0.015-0.25 mg/kg/hr Hard max: 1.67 mg/hr	DO NOT TITRATE		Sildenafil is compatible with D5W but MUST run in dedicated line
SOTALOL	Loading: 1 mg/kg IV given over 1 hour followed by continuous IV infusion	4.5 mg/kg/DAY Hard max: 12 mg/kg/DAY	DO NOT TITRATE		IV sotalol is compatible with D5W and NS but MUST run in a dedicated line
TERBUTALINE	Loading: 2-10mcg/kg IV over 5 minutes	0.08-4 mcg/kg/min Hard max: 4 mcg/kg/min	DO NOT TITRATE	Common side effects: tachycardia, HTN, tremor, N/	

				V, seizures	
TREPROSTINIL (REMODULIN®)	NONE	0.625-40 ng/kg/ min Starting dose: 0.625-1.25 ng/ kg/min; increased based on patient response Typical dose increase by 1.25-2.5 ng/kg/ min 1 ng (nanogram) = 0.001 mcg	DO NOT TITRATE		• Remodulin IV syringes/ cassette must be changed every 48 hours • IV Tubing must be changed every 48 hours • MUST BE FILTERED with 0.22micron filter • Administer through a dedicated line • Diluted treprostinil is stable for 48 hours • Avoid abrupt withdrawal
VASOPRESSIN (CARDIAC SHOCK)	NONE	<50kg: 0.0001-0.003 unit/kg/min Hard max: 0.003 unit/kg/min ≥50kg: 1-5 units/ hr Hard max: 5 units/hr	DO NOT TITRATE	• Pediatric dosing in units/kg/min • Adult dosing in units/hr • NOTE: cardiac shock dosing and DI dosing are DIFFERENT	• Concentrations greater than 0.4 units/mL must go via central line • Vesicant – if extravasation occurs, call Pharmacy (48208)
VASOPRESSIN (DIABETES INSIPIDUS)	NONE	0.25-10 milliunit/ kg/hr Hard max: 40 milliunit/kg/hr	Do not titrate, infusion will be titrated per physician orders	• NOTE: cardiac shock dosing and DI dosing are DIFFERENT • Dosage is highly variable and is titrated based on serum and urine sodium and osmolality in addition to fluid balance and urine output	• Vesicant – if extravasation occurs, call Pharmacy (48208)
VECURONIUM	Loading: may give 0.1 mg/kg IV over 5-10 seconds prior to initiating continuous	0.5-2 mcg/kg/ min Hard max: 4 mcg/kg/min	Titrate by 0.5 mcg/kg/min every 30-60 minutes until ordered Train- of-Four or max		• Vesicant – if extravasation occurs, call Pharmacy (48208)

	infusion Bolus: May give 0.1 mg/kg IV over 5-10 seconds with each infusion increase		ordered dose achieved		• No diluent – straight drug
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Note: adult dosing refers to patients >50 kg

***Doses listed above are suggested maximum doses for patient safety; start at lowest dose when titrating unless clinical condition requires higher dose. Some clinical conditions may warrant the use of doses higher than those listed above.**

Last revised: 05/05/21

Approved: Pediatric Medication Safety: 2/2013, 2/2019, 12/2019, 08/2020, 4/2021

Pharmacy & Therapeutics Committee: 2/2013, 4/2019, 1/2020, 9/2020, 5/2021

Approval Signatures

Step Description	Approver	Date
DOPS Policy Coordinators	Tiffany Smith: Clinical Pharmacist Specialist	3/22/2023
DOPS Policy Coordinators	Scott Ciarkowski: Pharmacy Manager, Quality Improvement & Safety and	3/14/2023
PolicyStat Administrator Review	Natalie Plata: Clinical Policy Manager	3/13/2023
PolicyStat Administrator Review	Olivia Curl: Admin Specialist Inter Health	3/13/2023
Owner Approval	Courtney Doellner: Clinical Pharmacist Specialist–Pediatric Medicati	3/13/2023